

NEW

2026

Ethiopian Health Profession Licensing Examination(EHPLE)

INFORMATION BOOKLET

NURSING



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MINISTRY OF HEALTH-ETHIOPIA

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HEALTHIER CITIZENS FOR PROSPEROUS NATION



AAU-Testing Center

Message From the State Minister, Ministry of Health -Ethiopia



H.E Mrs. Frehiwot Abebe

Improving healthcare quality is a global priority for sustainable development, with high quality healthcare being a key component of universal health coverage. One strategy to maintain health care standards is through provision of health professionals' competency assessment. Consequently, in 2019, the Ministry of Health-Ethiopia, initiated the Ethiopian Health Profession Licensing Examination (EHPLE) for undergraduates in seven health disciplines, which has since expanded to include 17 health disciplines.

The main goal of this competency assessment is to identify health professionals with minimal competencies necessary to perform their duties safely and competently, thus enhancing the quality of health care services. This initiative is overseen by a dedicated Health and Health Related Institutions and Professionals' Regulatory Lead Executive Office (LEO), comprising four desks, which plays a pivotal role in strengthening the system and enabling the LEO to conduct the competency exam more extensively and with improved organization and quality.

It is important to note that this competency assessment differs significantly from traditional academic or employment examinations. Hence, this information booklet has been created to address the informational needs of both examinees and teaching faculty regarding the EHPLE. Additionally, it aims to facilitate the assessment process, while promoting transparency and ensuring the sustainability of the program.

The preparation of this booklet involved the collaboration of esteemed experts from various higher education institutions, AAU-IER, the Ministry of Health, JHPIEGO-Ethiopia, Amref, Health Professionals 'Associations, and the Ministry of Education. Their invaluable contributions are acknowledged with sincere gratitude, alongside appreciation for the Ministry of Health staff for their unwavering commitment and hard work throughout the project.

Acknowledgements

This Information Booklet for EHPLE is a contribution from several educators, researchers, students, and concerned individuals with a genuine interest to propel Ethiopia's medical and health sciences education forward.

The Ministry of Health is grateful for the contribution of many individuals and institutions in realizing this endeavor. Among these are Professional Associations, Student Association, Higher Education Institutions (both public and private), JHPIEGO-Ethiopia, AMREF/HWIP, MoE (Ethernet), UNFPA, AAU-IER and all HHRIPR LEOs staff.



Acronyms and Abbreviations

EHPLE	Ethiopian Health Profession Licensing Examination
ETA	Educational and Training Authority
HEIs	Higher Education Institutions
HHrIPR-LEO	Health and Health-related Institutions and Professionals Regulatory Lead Executive Office
HSTP-II	Health Sector Transformational Plan-II
MCQ	Multiple Choice Question
MOH	Ministry of Health
WHO	World Health Organization

Purpose of the Information Booklet

The EHPLE Information Booklet serves as a comprehensive guide for those individuals seeking information about the exam. It typically outlines basic information for candidate registration, exam development and administration processes and procedures, result notification, and the licensing process. It also includes information on the exam framework, i.e., the exam domain, sub-domain, content, process, and task, with sample exam items specific to each profession.

The publication of this Booklet is crucial for the following reasons:

- **Clarity and guidance:** It provides clear information about the exam by ensuring candidates understand the necessary information to prepare them.
- **Accessibility:** It serves as a readily accessible resource for individuals pursuing to take the exam, consolidating essential information in one document and facilitating easy access to necessary details. It also helps other stakeholders who might be interested in such resources.
- **Transparency:** It promotes transparency in the examination process and fosters trust among stakeholders about the exam.

In summary, the publication of this Booklet is essential for creating a transparent, standardized, and accessible framework that guides candidates through the EHPLE process.

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Definition of terms

- **Domain:** a broad category or area of knowledge or skills of a profession
- **Sub-domain:** a subset of a broader domain that focuses on knowledge or skills related to the overarching domain
- **Content:** a more specific subcategory, which is a breakdown of the sub-domain
- **Task:** the responsibility, knowledge, skill, and attitude of a junior undergraduate professional in an actual work environment
- **Process:** a systematic sequence of steps or actions designed to achieve a specific outcome
- **Learning outcome:** a clear and measurable statement that describes what the examinee is expected to know or be able to do
- **Relative emphasis:** the proportional importance or weight assigned to different content areas or categories within the assessment
- **Item:** a particular multiple-choice question
- **Item developer:** a subject matter expert responsible for writing test items or questions that make up the examination
- **Item reviewer:** a subject matter expert responsible for reviewing and refining the test items or questions that make up the examination
- **Standard setting:** a process of determining a cut-off point or passing score for an exam
- **Item difficulty index:** a statistical measure that indicates the proportion of examinees who answered a particular test item correctly
- **Discrimination index:** a statistical measure that evaluates how well a particular test item differentiates between high-performing and low-performing examinees
- **Admission paper:** a printout paper generated by the system after completing registration that contains the examinee's photo, QR code, and necessary information

1. Introduction

1.1. Background

Competency assessment is one of the strategies for controlling the standard of healthcare services provided in healthcare facilities. The World Health Organization (WHO) recommends all healthcare professionals to have necessary competencies. In Ethiopia, the Health Sector Transformational Plan-II (HSTP-II) states competency assessment of all graduates before joining the health workforce as one of the strategic initiatives.

The Ministry of Health (MOH) launched the Ethiopian Health Profession Licensing Examination (EHPLE) for undergraduates in 2019. The Health and Health-related Institutions and Professionals Regulatory Lead Executive Office (HHrIPR-LEO) of the Ministry of Health is given a mission to implement the Ministry's objective to achieve competency-related goals. It has the responsibility to ensure that the EHPLE meets technical, professional, and legal standards, and to protect the health, safety, and welfare of the public by assessing candidates' abilities to practice competently.

Currently, the exam is given for 17 health professions (Medicine, Nursing, Public Health, Pharmacy, Medical Laboratory Science, Anesthesia, Midwifery, Dental Medicine, Medical Radiology Technology, Environmental Health, Psychiatric Nursing, Pediatric and Child Health Nursing, Emergency and Critical Care Nursing, Human Nutrition, Surgical Nursing, Optometry, and Physiotherapy). Since its introduction until December 2025, a total of 204,811 examinees took the exam in 23 rounds.

1.2. The Rationale of EHPLE

One of the critical functions of the MoH is to guarantee the efficiency, quality, and equity of healthcare delivery and to protect the public from any undesirable consequences in healthcare delivery practices. As professionals' competence is a significant determinant of the quality of health, evaluation of health professionals' competence has now been given due attention. The licensing examination for health professionals serves as a crucial step to ensure that individuals entering the field meet specific competency standards. The sole aim of the competency assessment is to safeguard public health by verifying that health professionals have the minimal basic knowledge, attitude, and skill required to provide safe and effective care.

Licensing exams act as a preventive measure, ensuring that only competent professionals join the health workforce, which, in turn, contributes to reducing the occurrence of medical errors and enhancing overall patient safety. By setting standards through examinations, regulatory bodies strive to minimize the risk of medical errors caused by incompetence.

2. Key processes of EHPLE

EHPLE involves several key processes to ensure the quality and reliability of the examination.

2.1. Registration of candidates

EHPLE has a mandatory online registration system for both new and repeat candidates, which can be found at <http://hple.moh.gov.et>

Please note these important notes during registration.

New Test Takers:



- The list of eligible candidates from governmental and private Higher Education Institutions (HEIs) will be uploaded to MoH's online registration system by the registrar office of their respective HEIs.
- Candidates who completed their education abroad must bring the following documents in person to MoH in order for their names to be uploaded to the online registration system:
 - ✓ Equivalence document from ETA
 - ✓ Externship attachment completion letter after completing the externship assigned by the regulatory body
- Once the name of the candidate is uploaded to the system and registration has opened for the current exam round, the candidate must register at <http://hple.moh.gov.et> by uploading the necessary documents listed below.
 - ✓ a scanned original or temporary degree
 - ✓ a scanned national ID

Repeat Test Takers:



- Since the information about re-exam candidates already exists in the system, the candidate should register by directly going to <http://hple.moh.gov.et>. There is no need for re-exam candidates to upload their documents.

Both new and repeat candidates:



- After completing the registration, the candidate must download and print the admission paper by logging into his/her account using username and password, or national ID FAN.
- The candidate can change the exam center by logging into his/her account only during the registration period
- An application for center change is not allowed after the registration period is closed.

2.2. Task Analysis

The first step of exam development involves conducting a comprehensive task analysis study, which identifies the tasks, knowledge, skills, and abilities required from a junior undergraduate professional in the specific profession. The analysis is typically done through surveys, interviews, or observations of practitioners in the actual work environment, as well as through the Delphi method with subject matter experts.

2.3. Exam Blueprint

Based on the task analysis findings, the exam blueprint is prepared that outlines the content areas to be covered in the examination and the weight or emphasis given to each area. This ensures that the exam reflects the key competencies and knowledge needed for competent practice in that specific profession. Blueprint is the matrix or chart that shows the number and type of test questions represented across the topics in the content area, consistent with the learning outcome and relative weight of the test given to each content area. The blueprint also identifies the percentage weighting of cognitive dimensions as the level of competence tested in each knowledge domain.

Key components of blueprint are:

- Domain
- Sub-domain
- Content
- Task
- Process
- Learning outcome
- Assessment methods
- Assessment tools/instrument (test format)
- Relative emphasis (in percentage)

2.4. Item Development

The items are developed following specific guidelines to ensure clarity, relevance, and fairness. Subject matter experts with experience in the field are selected from HEIs to develop exam questions (items) that align with the exam blueprint. The exam questions focus mainly on “knows how” according to the competency level of the Miller's pyramid. The items are produced in a secure location on designated computers that are free from internet connectivity. The items are scenario-based and constructed with stem, lead-in, and four options/alternatives. All items will have a single-best-answer type of Multiple-Choice Question (MCQ) that addresses the learning outcome defined in each content area. Standard textbooks, updated guidelines, and standards are used as reference materials.

2.5. Item Review

Once developed, the items undergo a rigorous review process by item reviewers. The main purpose of the exam review process is to evaluate content relevance, technical accuracy, clarity, and sensitivity related to culture and religion. More experienced subject matter experts as well as psychometric experts will do the review to ensure the items meet psychometric standards. Subject matter experts shall review the items to confirm that they are accurate, clearly stated, and correctly keyed using the checklist. Psychometric experts shall review the items to ensure that

they are not technically flawed. They also work on editorial review to check grammar, punctuation, and spelling errors. This helps ensure the reliability and validity of the items.

2.6. Standard setting method

The standard setting or cut-off point of the EHPLE is determined using the Modified-Angoff method, which is one of the most widely used and legally defensible standard setting approaches to set a cut-off point for high-stake competency examinations.

The method involves a panel of subject matter experts who evaluate each exam question and then estimate the probability that a minimally competent examinee would answer each test item correctly. The average of the experts' predictions for an exam question becomes its predicted difficulty. The average of the predicted difficulty values across all items on a test is the recommended cut-off point. This point indicates the minimum level of knowledge and skill required to pass.

2.7. Exam Administration

The EHPLE is administered following established protocols and guidelines. Proper exam administration procedures, appropriate security measures, and appropriate consideration for test-takers who need special support will be applied during exam administration at exam centers. The exam is administered in selected HEIs nationally, where candidates can choose based on their convenience at the time of registration. The exam schedule will be posted ahead of time on the MoH website and social media pages. Examinees who have a valid admission paper are eligible to sit for the exam. The mode of exam administration is computer-based testing.

CAUTIONS

- Candidates are allowed



- Attend the orientation session in order to sit for the exam
- Arrive at the exam center on time
- Bring a legal ID and admission paper
- Complete the exam within the allotted time frame

- Candidates are **NOT** allowed



- To bring reference materials, blank paper, or notes into the exam center
- To smoke, eat, or drink in the exam room
- To bring mobile phones, tablets, smart watches, camera devices, eyeglasses, calculators or any type of electronic device into the exam center
- To bring their personal belongings to the exam center
- To bring weapons and sharp materials into the exam center
- To give or receive assistance to or from other candidates during the examination

2.8. Scoring and post exam analysis

Once the exam is completed, the scoring process begins. The exam scoring process involves computerized scoring using software.

Post-exam analysis is the process of analyzing examinees' responses to individual test items in order to assess the quality of the items and the exam as a whole. This phase helps to identify any poorly performing items that may need revision or removal from the exam. The item difficulty index, discrimination index, and reliability coefficient are elements of exam analysis.

2.9. Result notification and appeal management

After scoring and analysis, individual score reports are generated and provided to examinees through the website <http://hple.moh.gov.et>. After result notification, examinees can submit their appeal through phone or email within 10 working days after result notification.

2.10. Licensing

The list of examinees who passed the exam will be sent to regional and city administration regulatory bodies. A license is obtained from the regional/zonal health bodies where he/she permanently lives.

Requirements for professional licensing are:



- Passing the EHPLE
- Original or temporary degree
- Educational documents (10th and 12th certificates)
- Medical certificate
- Government issued ID
- Additional prerequisites based on the requirements of regional regulatory bodies

3: Exam Framework

The key broader professional roles, also known as domains or main knowledge areas, serve as a building framework for the licensing examination content for *Nursing professionals*. The domains are further divided into discrete professional attributes that constitute sub-units (also referred to as sub-domains), defining the professional identity of nursing *professionals*. specifying the performance level of each sub-domain serves as the final characteristic of the professional duties on which the licensing exam focuses.

The contents of the licensing examination are presented below, structured into key roles (domains), sub-units (sub-domains), and tasks. The examination emphasis for each domain and sub-domain, out of the total 100% questions, is indicated in brackets.

Key professional roles/ domains

- ☐ Patient Care (80.0%)
- ☐ Scholar (4.0%)
- ☐ Professionalism (7.0%)
- ☐ Leadership and management (4.0%)
- ☐ Health promotion and disease prevention (5.0%)

Key role/ domain 1: Patient care (80.0%)

Description: This domain encompasses the professional roles of nurses in the provision of high-quality, safe, and patient-centered holistic nursing care within their scope of practice. The provision of up-to-date, ethical, and resource-efficient nursing care requires the application of integrated knowledge of biomedical, fundamental nursing, clinical, behavioral, and social sciences. As patient care providers, nurses shall implement the nursing care process by collecting and interpreting information, making clinical decisions, carrying out diagnostic and therapeutic nursing interventions, providing nursing care, and evaluating interventions. To demonstrate competence in this domain, candidates shall apply such integrated knowledge in the following sub-areas:

- ☐ Basic Nursing Procedures (25.0%)
- ☐ Adult Health Nursing (24.0%)
- ☐ Child Health Nursing (14.0%)
- ☐ Maternal Health Nursing (13.0%)
- ☐ Mental Health Nursing (4.0%)

Key role/ domain 2: Scholar (4.0%)

Description: This domain encompasses the professional roles of *Nursing professionals* in generating and utilizing scientific data to improve the health and well-being of the community and broaden their scientific knowledge within the *healthcare system and community setting*. Providing this service requires the application of integrated knowledge in research methods, measurements of health and disease, biostatistics, epidemiology, clinical audit, evidence-based practice, and research ethics. To demonstrate competence in this domain, candidates must possess applied knowledge in planning, problem

identification, data collection, analysis, interpretation, report write-up, and dissemination of research outputs.

Key role/ domain 3: Professionalism (7.0%)

Description: This domain encompasses the professional commitment of *Nursing professionals* to promoting the health and well-being of individuals and society through adhering to ethical standards, maintaining personal integrity, and upholding high standards of competence in all areas of *practice*. To exhibit competence in this domain, candidates must possess applied knowledge of ethical principles, medico-legal practices, effective communication, accountability to the profession and society, maintenance of professional excellence and personal health, and professional values such as compassion, respect, integrity, honesty, altruism, and humility.

Key role/ domain 4: Leadership and management (4.0%)

Description: This domain encompasses the professional roles of *Nursing professionals* in envisioning a high-quality healthcare system through self-awareness, active participation in healthcare teams, leading teams, and managing health systems. Providing this service requires the application of integrated knowledge in continuous quality improvement, effective health system leadership, management, and healthcare ethics. To demonstrate competence in this domain candidate shall possess applied knowledge to plan, organize, staff, lead, execute, monitor, and control healthcare resources and activities.

Key role/ domain 5: Health promotion and disease prevention (5.0%)

Description: This domain encompasses the professional roles of *Nursing professionals* in enhancing the health and well-being of patients, communities, and the larger populations they serve through health advocacy, disease prevention, health promotion and the promotion of health equity. Providing this service takes an integrated understanding of determinants of health, health informatics, epidemiology, communicable disease control, health education.

Table 1 Exam content for the Nursing profession

Domain 1: Patient care		
Sub-domain 1.1: Basic Nursing Procedures		
Content	Process	Tasks
Ear, Eye, and Nose Disorder	Pre-procedure assessment (Plan)	Assess and prepare a patient for ENN procedures
	Implementation	Perform ENN procedures
	Post-procedure Evaluation	Monitor the patient after ENN procedures
Respiratory System Disorder	Pre-procedure assessment (Plan)	Assess and prepare a patient for Respiratory system diagnostic and therapeutic procedures
	Implementation	Perform diagnostic and therapeutic procedures for a patient with a respiratory system disorder
	Post-procedure Evaluation	Monitor the patient after diagnostic and therapeutic procedures
Cardiovascular and Hematology System Disorder	Pre-procedure assessment (Plan)	Assess and prepare a patient for a cardiovascular disorder
	Implementation	Perform therapeutic cardiovascular procedures
	Post-procedure Evaluation	Monitor the patient with therapeutic cardiovascular procedures
Gastrointestinal System Disorder	Pre-procedure assessment (Plan)	Assess and prepare a patient for therapeutic and diagnostic GI procedures
	Implementation	Perform therapeutic and diagnostic GI procedures
	Evaluation	Monitor Patient with therapeutic GI procedures
Genito Urinary System Disorder	Pre-procedure assessment (Plan)	Assess and prepare a patient for Genitourinary system diagnostic and therapeutic procedures
	Implementation	Perform genitourinary system diagnostic and therapeutic procedures
	Post-procedure Evaluation	Monitor patient with Genitourinary system diagnostic and therapeutic procedures
Integumentary System Disorder	Pre-procedure assessment (Plan)	Assess and prepare a patient for therapeutic procedures for Integumentary System Disorder
	Implementation	Perform therapeutic procedures for Integumentary System Disorder
	Post-procedure Evaluation	Monitor patient with therapeutic procedures with Integumentary System Disorder
Musculoskeletal System Disorder	Pre-procedure assessment (Plan)	Assess and prepare the patient for therapeutic procedures for Musculoskeletal System Disorder
	Implementation	Perform therapeutic procedures for Musculoskeletal System Disorder
	Post-procedure Evaluation	Monitor patient with therapeutic procedures for Musculoskeletal System Disorder
Sub-domain 1.2: Adult Health Nursing		
Content	Process	Tasks
Ear, Eye, and Nose Disorder (Conjunctivitis, Glaucoma, cataract, Otitis media, trachoma)	Assessment	Assess a patient with EEN disorders
	Diagnosis	Diagnosis for a patient with EEN disorders
	Planning and Implementation	Develop a nursing care plan and manage patients with EEN disorders

	Evaluation	Monitor patients' response for management of EEN disorders
Respiratory system disorders (Airway obstruction, tonsillitis, acute bronchitis, TB, Bronchial Asthma, pneumonia, pulmonary Embolism)	Assessment	Assess a patient with respiratory disorders
	Diagnosis	Diagnose patients with respiratory disorders
	Plan and Implementation	Develop a nursing care plan and manage a patient with respiratory disorders
	Evaluation	Monitor patients with respiratory disorders for treatment response.
Cardiovascular and Hematology system Disorder (Shock, hypertension, angina pectoris, MI, heart failure, anemia, DVT and varicose veins)	Assessment	Assess and make a nursing diagnosis for a patient with Cardiovascular disorders
	Diagnosis	Diagnose a patient with Cardiovascular disorders
	Planning and Implementation	Develop a nursing care plan and manage a patient with a cardiovascular system
	Evaluation	Monitor patients' response to treatment of cardiovascular system disorders
Gastrointestinal system disorders (PUD, Gastritis, Constipation, diarrhea, Acute abdomen, Hemorrhoid, BO, liver cirrhosis, pancreatitis, and Cholecystitis)	Assessment	Assess a patient with Gastrointestinal disorders
	Diagnosis	Diagnose a patient with Gastrointestinal disorders
	Planning and Implementation	Develop a nursing care plan and manage patients with GI disorders
	Evaluation	Monitor patients' response to the treatment of the GI disorder
Endocrine Disorder (DM, Thyroid, parathyroid)	Assessment	Assess a patient with endocrine system disorders
	Diagnosis	Diagnose a patient with endocrine system disorders
	Planning and Implementation	Develop a nursing care plan and manage a patient with endocrine system disorders
	Evaluation	Monitor patients' response for treatment of complications
Genito Urinary System Disorder (Glomerulonephritis, Urinary tract infections, Renal failure, BPH, Urolithiasis, fluid and electrolyte imbalances, acid-base imbalances)	Assessment	Assess a patient with genitourinary system disorders
	Diagnosis	Develop a nursing diagnosis for a patient with genitourinary system disorders
	Planning and Implementation	Develop a nursing care plan and manage a patient with genitourinary system disorders
	Evaluation	Monitor patients' response to treatment, Genitourinary system disorders
Immune System Disorder (HIV, hypersensitivity disorders)	Assessment	Assess a patient with immune system disorders
	Diagnosis	Diagnosing a patient with an immune system disorder
	Planning and Implementation	Develop a nursing care plan and manage patients with immune system disorders
	Evaluation	Monitor patients with immune system disorders
Integumentary system Disorder (Burn, Viral, Bacterial, fungal, and dermatitis)	Assessment	Assess a patient with Integumentary system disorders
	Diagnosis	Diagnose a patient with common integumentary disorders
	Planning and Implementation	Develop a nursing care plan and manage patients with common Integumentary system disorders
	Evaluation	Monitor patients' response for treatment of integumentary disorders
Neurologic system problem (Cerebrovascular accident, increased	Assessment	Assess a patient with neurologic disorders
	Diagnosis	Diagnose a patient with neurologic disorders
	Planning and Implementation	Develop a nursing care plan and manage a patient with neurologic disorders

ICP, neurologic trauma, meningitis, headache, seizure disorders)	Evaluation	Monitor patients' responses for the treatment of neurologic disorders
Musculoskeletal system disorders (amputation, fracture, osteoarthritis, rheumatoid arthritis)	Assessment	Assess a patient with musculoskeletal system disorders
	Diagnosis	Develop a nursing diagnosis for a patient with common musculoskeletal system disorders
	Planning and Implementation	Develop a nursing care plan and manage patients with musculoskeletal system disorders
	Evaluation	Evaluate a patient with musculoskeletal system disorders
Perioperative nursing care	Assessment	Assess the patient with perioperative nursing care
	Diagnosis	Develop a nursing diagnosis for a perioperative patient
	Planning and Implementation	Develop a nursing care plan and manage perioperative patients
	Evaluation	Monitor perioperative patient condition
Communicable Disease (Acute febrile illness, Rabies, Anthrax, Covid-19, cholera, amebiasis, giardiasis, Ascariasis, hepatitis, Hookworm disease, and STIs)	Assessment	Assess patients with communicable diseases
	Diagnosis	Diagnose the patient with a communicable disease
	Planning and Implementation	Develop a nursing care plan and manage a patient with a communicable disease
	Evaluation	Monitor patients' responses to communicable diseases
Subdomain 1.3: Child Health Nursing		
Content	Process	Tasks
Growth, Development, and Malnutrition	Assessment and Diagnosis	Assess and diagnose the growth, development, and nutritional status of a child
	Planning and Implementation	Develop a nursing care plan and manage a child with growth, development, and nutrition problems
	Evaluation	Monitor response to management of growth, development, and nutrition problems
Neonatal care (Birth injuries, low birth weight, preterm, Neonatal sepsis, Neonatal tetanus, neonatal jaundice, hypothermia, Hypoglycemia, Birth asphyxia)	Assessment and Diagnosis	Assess and diagnose a neonate with neonatal problems
	Planning, Implementation, and Evaluation	Develop a nursing care plan, manage, and monitor neonatal problems
Immunization (Estimating vaccine needs, Calculation of target population, EPI coverage, types of vaccine, and vaccine schedule)	Assessment	Assess the child for immunization
	Implementation	Manage vaccination and vaccines
IMNCI	Diagnosis	Assess and classify a child aged below two months and a child aged two to 59 months using the IMNCI protocol
	Intervention	Treat a child aged below two months and a child aged two to 59 months using the IMNCI protocol
	Monitor	Follow a child aged below two months and a child aged two to 59 months using the IMNCI protocol
Respiratory system disorders (tonsillopharyngitis, croup, pertussis)	Assessment and Diagnosis	Assess and diagnose a child with respiratory system disorders
	Planning, Implementation, and Evaluation	Develop a nursing care plan, manage, and monitor a child with respiratory system disorders

Cardiovascular system Disorders (rheumatic fever, congestive heart failure)	Assessment and Diagnosis	Assess and diagnose a child with cardiovascular disorders
	Planning and Evaluation	Develop a nursing care plan and evaluate a child with a cardiovascular disorder
GIT system Disorders (appendicitis, intussusception)	Assessment and Diagnosis	Assess and diagnose a child with gastrointestinal disorders
	Planning and evaluate	Develop a nursing care plan and monitor a child with gastrointestinal disorders
Endocrine Disorders (diabetes mellitus)	Assessment and Diagnosis	Assess and diagnose a child with Endocrine Disorders
	Planning, Implementation, and Evaluation	Develop a nursing care plan, manage, and monitor a child with Endocrine Disorders
GUT system Disorders (urinary tract infection, acute glomerulonephritis, nephrotic syndrome)	Assessment and Diagnosis	Assess and diagnose a child with genitourinary system disorders
	Planning, implementation, and evaluation	Develop a nursing care plan, manage, and evaluate a child with genitourinary system disorders
Integumentary system disorders (burns, eczema, cellulitis, Impetigo, Scabies)	Assessment and Diagnosis	Assess and diagnose a child with an integumentary system disorder
	Planning, implementation, and evaluation	Develop a nursing care plan, manage, and monitor a child with an integumentary system disorder
Neurologic Disorders (febrile seizure disorders and Meningitis)	Assessment and Diagnosis	Assess and diagnose a child with neurologic disorders
	Planning and evaluate	Develop a nursing care plan and monitor a child with neurologic disorders
Musculoskeletal System Disorder (Osteomyelitis and septic arthritis)	Assessment and Diagnosis	Assess and diagnose a child with musculoskeletal system disorders
	Planning, Implementation, and Evaluation	Develop a nursing care plan, manage, and monitor a child with a musculoskeletal disorder

Subdomain 1.4: Maternal Health Nursing

Content	Process	Tasks
ANC (APH, PIH, GDM, Anemia during pregnancy, hyperemesis gravidem during pregnancy)	Assessment	Assess the pregnant woman
	Diagnosis	Diagnose a pregnant woman
	planning	Develop a nursing care plan for pregnant woman with abnormal pregnancy
	Implementation	Manage pregnant woman with abnormal pregnancy
	Evaluation	Monitor pregnant woman with abnormal pregnancy
Labor and Delivery	Assessment	Assess a pregnant woman with labor
	Diagnosis	Diagnose laboring woman
	plan	Develop nursing care plan of laboring woman
	Implementation	Manage a woman with labor
	Evaluation	Monitor a pregnant woman with labor
Postnatal care	Assessment	Assess a mother in postnatal period
	Diagnosis	Diagnose a postnatal woman in post-natal period
	Implementation	Provide essential newborn care
	planning	Develop postnatal nursing care for postnatal mother
	Implementation	Manage a mother and newborn in postnatal period
	Evaluation	Monitor postnatal mother in postnatal period
Family Planning	Assessment	Assess a woman for family planning service
	Implementation	Provide family planning
	Evaluation	Monitor a woman using family planning

Gynecologic care (Dysmenorrhea, amenorrhea, polymenorrhea, oligomenorrhea, abortion)	Assessment	Assess a woman with bleeding
	Diagnosis	Diagnose a woman with bleeding
	planning	Develop nursing care plan for a woman with bleeding
	evaluation	Monitor a woman treated for bleeding
Subdomain 1.5: Mental Health		
Content	Process	Tasks
Mental Illnesses (schizophrenia spectrum, Major depressive disorder, anxiety, and related disorders)	Assessment and Diagnosis	Assess and diagnose patients with mental illnesses and disorders
	Planning and Implementation	Develop a nursing care plan and manage patients with mental illnesses
	Evaluation	Monitor the progress of patients in treatment for mental illnesses
Trauma and stress- related Disorders (post-traumatic stress disorder, acute stress disorder)	Assessment and Diagnosis	Assess and diagnose a patient with trauma and stress-related Disorders
	Planning, Implementation, and Evaluation	Develop a nursing care plan, manage and monitor patients with trauma and stress-related Disorders
Substance-related and Addictive Disorder (alcohol, chat, and cigarette use disorder)	Assessment and Diagnosis	Assess and diagnose a patient with a substance-related and Addictive Disorder
	Planning and Implementation	Develop a nursing care plan and manage a patient with substance-related and addictive disorders
	Evaluation	Monitor the progress of patients with substance-related and addictive disorders
Domain 2: Health promotion and disease prevention		
Task		
Assess and diagnose community health status		
Intervene in community health problems		
Domain 3: Management and Leadership		
Task		
Apply basic managerial functions		
Create a safe and conducive environment for clients and staff		
Lead nursing team		
Domain 4: Scholar		
Task		
Develop a research proposal		
Analyze and interpret quantitative data and information		
Domain 5: Professionalism		
Task		
Adhere to legal and ethical principles about nursing practice (including ethical principles and ethical dilemmas)		
Establish and maintain a positive, respectful, and collaborative working relationship with clients, client families, the nursing team, and other working colleagues		
Produce and maintain accurate documentation of activities, procedures, and results		
Apply therapeutic communication and demonstrate professionalism		

Sample questions

1. A 40-year-old female patient was admitted to a medical ward after she is presented with generalized edema, fever and headache. She had history of treatment for tonsillitis four weeks back. On urinalysis, her urine is brown in color and she has proteinuria of 3.7g/day.

What is the most likely diagnosis of the patient?

- | | |
|------------------------|--------------------|
| (A) Glomerulonephritis | (C) Pyelonephritis |
| (B) Nephrotic syndrome | (D) Urolithiasis |

Answer Key: The answer is **B**

Explanation. In this scenario the patient was presented with generalized edema, fever, headache and proteinuria which are the general manifestation of glomerular diseases. Generally, loss of up-to 150 mg/day of protein in urine is normal. Protein loss greater than 2.5g/day in urine is usually an indication of glomerular diseases. But, proteinuria greater than 3.5gm/day is typical indication of nephrotic syndrome. Whereas, proteinuria between 2.5-3.5g/day is also indication of glomerulonephritis. Patients with both nephrotic syndrome and glomerulonephritis have edema but edema in patients with nephrotic syndrome is generalized. Fever and headache are non specific symptoms. Patients with glomerular diseases may have Brown to black urine color but not specific. There is nothing mentioned in this case scenario that indicates urolithiasis.

2. A 48-year-old male client visited outpatient department with complaint of chest pain and is diagnosed for atherosclerosis. On history, client used one pack of cigarette per day for the last ten years and he was a finance officer for a long period of time. The patient has no finding on laboratory investigation.

What is the most appropriate management for this client?

- | | |
|-----------------------------|---------------------------------|
| (A) Cessation of smoking | (C) Reduce duration of seating |
| (B) Treating hyperlipidemia | (D) Prescribe high dose aspirin |

Answer Key: The answer is **A**

Explanation: in this case the patient visits the hospital with complaint of chest pain secondary to atherosclerosis. The management of atherosclerosis involves modification of risk factors, a controlled exercise program to improve circulation, anti-hyperlipidemic agents, thrombolytic and interventional or surgical graft procedures. But in this case the laboratory investigation was normal so we don't have any evidence to start anti hyperlipidemic agents and anti thrombolytic drugs. The only risk factor that was mentioned in the case scenario was smoking more than one pack of cigarette per day. The best method of treatment for this patient is modification of risk factors. A nurse needs to engage the patient in smoking cession program.

3. A 36-year-old female client visited a clinic for a routine cardiovascular risk assessment. She was tested for underlying metabolic abnormalities. The patient has fasting blood sugar level of 130mg/dl, Blood pressure 130/80 mmHg, waist circumference 36 cm and hip circumference 67cm.

What examination finding of the patient is suggestive for her metabolic abnormality?

- (A) Fasting plasma glucose (C) Waist to hip ratio
(B) Waist circumference (D) Blood pressure

Answer Key: The answer is A

Explanation: In this scenario the patient came for cardiovascular risk assessment. To diagnose whether or not a patient has a metabolic syndrome, at least three of the following criteria should be fulfilled. These include insulin resistance (fasting plasma glucose more than 100 mg/dL or abnormal glucose tolerance test), central obesity (waist circumference more than 35 inches in women, more than 40 inches in men), dyslipidemia (triglycerides more than 150 mg/dL, HDL less than 50 mg/dL in women, less than 40mg/dL in men), Blood pressure persistently >130/85 mmHg, proinflammatory state (high levels of C-reactive protein) and Prothrombotic state (high fibrinogen level). In this case scenario the finding that suggests the patient has metabolic abnormality is fasting blood glucose level of 130mg/dl.

4. A 29-year-old man was presented to an ophthalmologic OPD with complaints of difficulty of focusing and blurred vision. The nurse utilized Snellen chart to assess visual acuity and the finding was 20/70.

What is the most likely diagnosis for this finding?

- (A) Normal vision (C) Total blindness
(B) Legal blindness (D) Visual impairment

Answer Key: The answer is D

Explanation: In this scenario the nurse tried to assess the visual acuity of the patient by using Snellen's chart. This technique is used to assess the patient's ability to see things from far and near. Normal vision is 20/20. Visual impairment is any kind of vision loss. Legal blindness is 20/200. An example of findings is that the patient who identifies all the letters correctly on the line marked 30 has a visual acuity of 20/30. This means that the patient can see at 20 feet what the average individual can see at 30 feet.

5. A 38-year-old female patient came to health center with complaint of abdominal pain, vaginal discharge and severe pruritis. Microscopic examination of vaginal discharge revealed pseudohyphae.

What is the most likely explanation for this finding?

- (A) The patient has genital ulcer
- (B) The patient has genital herpes
- (C) The patient has trichomonas vaginalis
- (D) The patient has vulvo vaginal candidiasis

Answer Key: The answer is **D**

Explanation: In this scenario the patient comes with abdominal pain and vaginal discharge. So the possible causes of vaginal discharge include Neisseria gonorrhea, Chlamydia trachomatis, Trichomonas vaginalis, Gardnerella vaginalis (Polymicrobial) and Candida albicans . Patient with vulvo vaginal candidiasis is characterized by severe vulvar pruritis (prominent feature), vaginal discharge (scanty, whitish, yellow, thick to form curds, non-offensive, sore vulva due to itching. From the scenario the patient has abdominal pain, vaginal discharge and Microscopic examination showed pseudohyphae which are the etiologic agent of vaginal candidiasis.

6. A mother, who has been lactating for six months, came to a family planning clinic upon return of her menstrual period. The nurse discussed with her about contraception options, their side effects and benefits. After discussion the mother preferred the injectable method of contraception.

What is the appropriate route to administer the contraceptive she chose?

- (A) Intra dermal
- (B) Intra muscular
- (C) Subcutaneously
- (D) Intra venous

Answer Key: The answer is **B**

Explanation: In this scenario the major intention of the question is to select appropriate route of administration of Depo-Provera. It can be administered through intramuscular and subcutaneously, but it cannot be given through intravenous route because the absorption should be slow. Depo-Provera should be absorbed slowly over 3 months. Most of the time the drug administered through subcutaneously should not be greater than 1 ml and the actual dose of Depo-Provera is 2ml. which can't be administered through subcutaneously.

7. A 46-year-old man, with a history of excessive alcohol consumption for ten years, was hospitalized because of seizure and increased hand tremor. Symptoms follow three days after cessation of alcohol as part of his repeated unsuccessful effort to stop alcohol consumption. He has no identified medical illness.

What is the most likely admission diagnosis for this patient?

- | | |
|----------------------------|--------------------------|
| (A) Alcohol abuse disorder | (C) Alcohol withdrawal |
| (B) Alcohol dependency | (D) Alcohol intoxication |

Answer key: The answer is C

Explanation: This patient has been heavy consumer of alcohol for long time. The patient's manifestation and the time in which the manifestation occurred are more related to the possible diagnosis of alcohol withdrawal. According to DSM-5, Alcohol withdrawal is diagnosed using two of the four criteria which include cessation of alcohol, development of sign or symptom short after cessation of alcohol. Symptoms are not related to other medical illness and the symptoms cause significant impairment in normal functioning of the patient. Alcohol abuse and alcohol intoxication are related with recent intake of alcohol, and a strong desire or urge to use alcohol.

8. A premature neonate was admitted to neonatal intensive care unit for the treatment of very low birth weight and inability to gain weight for the first three weeks. The neonate has weak suckling reflex, absence of grasping and tonic neck reflexes.

What is the most appropriate nursing diagnosis of this case?

- | | |
|----------------------------|------------------------------------|
| (A) Impaired parenting | (C) Ineffective breast feeding |
| (B) Altered growth pattern | (D) Delayed growth and development |

Answer key: The answer is C

Explanation: In this scenario, the objective and subjective data for the neonate are low birth weight, inability to gain weight and weak suckling reflex and absence of grasping. Based on these, a nurse can formulate a nursing diagnosis of ineffective breast feeding related to weak sucking reflex and Risk for altered growth pattern related to low birth weight. To formulate a nursing diagnosis of impaired parenting, the nurse must further identify maternal knowledge and effort to breast feed. Neonates with asymmetric tonic neck reflex and uncoordinated hand movement in the first month of life are in normal Growth and developmental stage.

9. The nurse evaluates a two-year-old child who was admitted for the treatment of failure to thrive secondary to severe acute malnutrition. The child had good appetite and no edema for the last ten consecutive days. The nurse decided to discharge the child.

What is the most likely additional finding that could support the nurse's decision?

- (A) Weight for age >85% (C) Weight for height >80%
(B) Weight for age >90% (D) Weight for height > 85%

Answer Key: The answer is **D**

Explanation: For a child admitted in phase two severe acute malnutrition treatments, the nurse can make evaluation to discharge or transfer the child. To discharge a child aged from 6 to 59 months and who completed phase two treatment, the child must have weight for length greater or equal to 85% and no edema for 10 days. The nurse can also measure the weight achieved if the child has been admitted with MUAC criteria. In contrary, weight for age measurement is not sufficient enough to indicate the child's health status during severe acute malnutrition treatment evaluation.

10. A two-year-old male baby was brought to a health center with a complaint of barking cough, which is aggravated during night time and when crying. Physical examination result revealed persistent low-grade fever and slightly increased respiratory rate. He has no drooling and pain during swallowing.

What is the most likely diagnosis of this child?

- (A) Whooping cough (C) Croup
(B) Epiglottitis (D) Pertussis

Answer key: The answer is **C**

Explanation: The chief complaint of this child, specifically barking cough and low grade fever mainly suggest the possible diagnosis of Croup. A child with croup can be further identified by slight increment of respiratory rate and often recurrence with decreasing intensity for several days. Epiglottitis can be differentiated by progressive and fulminating course of high fever with sore throat, difficulty in swallowing and dyspnea. Whooping cough or Pertussis mainly manifest by congestion and rhinorrhea with sneezing and lacrimation in the Catarrhal stage. The cough of pertussis is characterized by dry, intermittent, bursting and uninterrupted on single exhalation.

11. A seven-year-old child was brought to a pediatric OPD with a complaint of hematuria. On examination, the patient had edema, hypertension and oliguria. The clinician wanted to know the possible risk factor for this case.

What is the most likely underlying cause for the disease?

- (A) History of recurrent croup
- (B) History of recurrent pneumonia
- (C) History of recurrent tuberculosis
- (D) History of recurrent tonsillo-pharyngitis

Answer key: The answer is **D**

Explanation: A child in age group of 5-12 years with hematuria can be presented with a number of additional symptoms to indicate specific diseases. For this child, the hematuria is accompanied by edema, hypertension and renal insufficiency which make the most likely diagnosis to be post streptococcal glomerulonephritis. Previous recurrent history of streptococcal infection of the tonsils or pharynx is one of the causative factors that predispose the child for glomerulonephritis. This recurrent infection results in immune mediated complex formation, complement activation glomerular injury, and hematuria. However, pneumonia, tuberculosis and croup are not associated with immune mediated antigen antibody complex formation.

12. A flood disaster occurred and high morbidity and mortality led to mass displacements of populations from the area in a woreda. After the incident, disaster preparedness and management team wanted to identify the immediate factors which led to highest morbidity and mortality in the area.

What is the most likely disaster consequence in this scenario?

- (A) Child abuse
- (B) Diarrheal illness
- (C) Mental health disorders
- (D) Sexual transmitted disease

Answer key: The answer is **B**

Explanation: During flood disaster, the community suffers from multiple causes resulting in displacement of the population. In this community water-borne disease related to hygiene and shortage of clean water supply is an immediate factor that led the population to high morbidity, mortality and displacement. Diarrheal illness is one of the commonest water related illness that mostly occurs following flooding. Because of its severity and communicability, diarrheal disease requires an immediate intervention. Displacement of the population due to child abuse, sexual

transmitted disease and mental disorder, are rare during disaster and the fatality and severity of these problems do not exceed diarrhea.

13. In a cardiac center of a hospital, more than 5000 children and other cardiac patients are on waiting list for cardiac surgery because of resource scarcity. Along with solving the budget constraint and training of health professionals, the hospital management team decided to give service in their order of visit.

What is the most appropriate principle applied in this context?

- | | |
|--------------|--------------------|
| (A) Justice | (C) Beneficence |
| (B) Autonomy | (D) Nonmaleficence |

Answer Key: The answer is A

Explanation: Health care personnel are expected to treat every client equally and ethically. In this scenario, the hospital management has been committed to render service based on order of visit. This means the hospital is fair for all clients. Justice is an act of treating all patients equally, irrespective of their sex, education, income or other personal background. Thus, the principle of justice is visibly applied in this scenario. Beneficence and non-maleficence are also ensured indirectly. On the other hand, the issue of autonomy is less entertained in the scenario.

14. A staff recruitment committee in a hospital designed sound human resource management system. During the course of planning, the committee anticipates changes in the present work force in terms of entry of new professionals and departures of the employees for different reasons regularly. Due to this the hospital has sufficient information about the number and mix of human resources.

What is the most appropriate step of human resource planning done by the committee?

- | | |
|----------------|------------------|
| (A) Profiling | (C) Forecasting |
| (B) Estimating | (D) Inventorying |

Answer Key: The answer is C

Explanation: Human resource management is the process of acquiring and retaining the organization's human resource. To acquire appropriate mix of human resource, human resource planning precedes the other acquiring activities to determine staffing needs. Human resource planning involves five steps; profiling (i.e. profile the personnel need of the organization at some future point), estimating (i.e. projecting the type and number of personnel needed), inventorying (i.e. human resource audit of present employees), forecasting (i.e. anticipating changes in the present work force in terms of entries and exits and the final step is ensuring the right number of personnel with appropriate prerequisite knowledge and skill.

15. A research finding on diarrhea status among children of 6-23 months showed 47% mild diarrhea, 36 % moderate diarrhea and 17% severe diarrhea. Finally, the researcher planned to present the data in graphs.

What is the most likely type of graph to be used by this researcher?

- (A) Bar graph (C) Histogram
(B) Line graph (D) Ogive curve



Answer Key: The answer is A

Explanation: Bar graph is one of the data presentation methods used for the categorical variables. The research finding on the diarrhea status was categorized into mild, moderate and severe categories which entails bar graph as the best method of presentation. But the other types of graphs are important for numeric type of variable presentation methods. Line graph is useful for presenting numeric variables over the passage of time; histogram is for continuous measurement variables; ogive curve similar with histogram to present cumulative frequencies of a distribution. Therefore, the correct answer for the question is option A.



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